

This Patient Group Direction (PGD) is intended only for registered healthcare professionals who have been named and authorised by their organisation to practice under it.

Patient Group Direction

for the administration of Nitrofurantoin 100mg MR capsules or Trimethoprim 200mg tablets for the treatment of Recurrent Uncomplicated Urinary Tract Infection (UTI) in Women

Summary of NICE / NICE CKS Guidelines for Recurrent Uncomplicated Urinary Tract Infection (UTI) in Women

Overview

- **Definition:** Recurrent UTI defined as ≥ 2 UTIs in 6 months or ≥ 3 in 12 months. Common in women. Most caused by E. coli.

Assessment

- Urinary symptoms: dysuria, frequency, urgency, suprapubic pain
- Urine dipstick: positive nitrites and/or leucocytes
- Duration of symptoms
- Previous antibiotic use and resistance patterns
- Pregnancy status
- Comorbidities and renal function

Management

- Antibiotic treatment per NICE guidelines
- Self-care advice: increase fluid intake, complete full course
- Consider prophylactic strategies for frequent recurrence

Red Flags - Refer to GP

- Fever or systemic symptoms (suggests pyelonephritis)
- Haematuria in older adults
- Catheter-related UTI
- Pregnant women
- Immunocompromised patients

Patient Group Direction

for the supply/administration of

Nitrofurantoin 100mg Modified-Release Capsules

for the treatment of

Recurrent Uncomplicated Urinary Tract Infection (UTI) in Women

Healthcare professionals covered and training

	Healthcare professionals allowed to work under this PGD and requirements
Qualifications and Registration	- Pharmacist registered and practicing with the GPhC - Pharmacy technician registered and practicing with the GPhC
Training and assessment	Pharmacists and Pharmacy Technicians must have completed training relevant to this condition to use this PGD. This training will be documented and overseen by the Get Real Health team. - All users must be familiar with the SPC for the products as well as BNF advice and any applicable national guidelines - All users must previously have used PGDs to supply medication - Must work in compliance with the SOPs of their own employer - All users must have appropriate indemnity in place to cover this service
Further training and responsibilities	- All users must be up to date with CPD requirements and appraisal - All users must be up to date with MHRA and safety alerts with regards to medical products - All users must understand the concepts of capacity and consent and be aware of the Mental Capacity Act 2005.

PGD Indication	Treatment of uncomplicated recurrent lower urinary tract infection (cystitis) in non-pregnant women aged 16-65 years
Inclusion criteria	Non-pregnant women aged 16-65 years Symptoms consistent with uncomplicated lower UTI (dysuria, urinary frequency, urinary urgency, suprapubic pain) Urine dipstick positive for nitrites and/or leucocytes History of recurrent UTI (≥ 2 in 6 months or ≥ 3 in 12 months) Valid informed consent obtained
Exclusion criteria	Pregnancy Breastfeeding (relative contraindication - small amounts excreted in breast milk) eGFR < 45 ml/min (increased risk of toxicity) Known hypersensitivity to nitrofurantoin or excipients G6PD (glucose-6-phosphate dehydrogenase) deficiency Acute porphyria Systemic symptoms or fever (signs of pyelonephritis - refer to GP) Haematuria

	Catheter-related UTI
Cautions	Renal impairment: use with caution if eGFR 45-60 ml/min Elderly patients (≥65 years) - use with caution, assess renal function Diabetes mellitus - increased infection risk Electrolyte imbalance Vitamin B deficiency or folate deficiency Pulmonary disease (asthma, COPD) - risk of pulmonary reactions Hepatic impairment Long-term use associated with risk of pulmonary, hepatic and peripheral nervous system reactions
Actions if patient is excluded or declines treatment	Advise on alternative treatment options and how these can be accessed. Document any advice given and the decision reached. Inform or refer to the GP as appropriate.

Details of the medicine

Name, form and strength of medicine	Nitrofurantoin 100mg modified-release capsules
Legal category	Prescription Only Medicine (POM)
Storage	Store below 25°C. Protect from light.
Route/method of administration	Oral - via mouth
Dose and frequency	100mg twice daily for 3 days (acute uncomplicated UTI) 100mg twice daily for 5-7 days if symptoms persist after 3 days
Quantity to be administered and/or supplied	6 capsules (3-day course) or up to 14 capsules (7-day course)
Maximum or minimum treatment period	3-7 days maximum
Adverse effects	Common: nausea, vomiting, diarrhoea, anorexia Uncommon: peripheral neuropathy, pulmonary reactions, hepatic reactions Rare: chronic pulmonary fibrosis, cholestatic jaundice, megaloblastic anaemia, haemolytic anaemia

Yellow card reporting	Report suspected adverse effects via https://yellowcard.mhra.gov.uk and inform the GP as appropriate.
Records to be kept	Record the following information:

	• that valid informed consent was given • name of individual, address, date of birth and GP • name of healthcare practitioner • name and brand of medication • date of supply dose, form and route quantity supplied • advice given, including advice given if excluded or declines treatment • details of any adverse drug reactions and actions taken supplied via PGD Records should be signed and dated. All records should be clear, legible and contemporaneous. Electronic records can be made. A record of all individuals receiving treatment under this PGD should also be kept for audit purposes as follows: • Adults aged 18 years and over - keep records for 8 years • Children aged under 18 years - keep records until the 25th birthday (if the patient was 17 years when treatment was finished, keep records until the 26th birthday)
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Patient information

Written information to be given to patient or carer	Supply the patient information leaflet (PIL) provided with the medication.
Follow-up advice to be given to patient or carer	Complete the full course of antibiotic treatment even if symptoms improve Increase fluid intake to help flush the urinary system Seek medical advice if symptoms worsen or do not improve within 48 hours Seek medical advice if fever develops or systemic symptoms occur Report any adverse effects or allergic reactions to healthcare professional Do not share medication with others Store medication as directed on the container

Key references

• NICE Medicines Practice Guideline 2 (MPG2): Patient Group Directions. Published March 2017 https://www.nice.org.uk/guidance/mg2 • NICE MPG2 Patient group directions: competency framework for health professionals using patient group directions. Updated March 2017 https://www.nice.org.uk/guidance/mpg2/resources • NICE CKS Guidance: Urinary tract infections: Recurrent uncomplicated urinary tract infection in women • British National Formulary (BNF) Key information on the selection, prescribing, dispensing and administration of medicines

Agreement to practise by the Registered Healthcare Professional

By signing this PGD you are confirming that you agree to its contents and that you will work within it. PGDs do not remove inherent professional obligations or accountability. It is the responsibility of each healthcare professional to practise only within the bounds of their own competence and professional code of conduct.

Name and address of pharmacy premises / clinic premises to which this PGD relates			
Name of healthcare professional	Job title	Registration number	Signature
Valid from date	Expiry date		
1/11/25	31/10/26		

PGD adoption and authorisation by the employer/clinical lead

The employer has ensured that the person using this PGD, has the knowledge and skills to safely provide the service which will encompass the supply or administration of a medication or medicine.

This section must be signed by the superintendent or clinical lead responsible for this service.

Name of superintendent / clinical lead	Job title & name of organisation	Signature	Date
Name of professional group signatory	Job title & name of organisation	Signature	Date

	(GPhC: 2046322)		
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Signed on behalf of Get Real Health

Name	Qualification	Signature	Date
Nitin Shori	Medical Doctor GMC: 6047293		2/11/25
Chris Pilkington	Pharmacist GPhC: 2046322		2/11/25

Change history

Version number	Change details	Date
001	Development & issue of new PGD	1st November 2025

Patient Group Direction

for the supply/administration of

Trimethoprim 200mg Tablets

for the treatment of

Recurrent Uncomplicated Urinary Tract Infection (UTI) in Women

Healthcare professionals covered and training

	Healthcare professionals allowed to work under this PGD and requirements
Qualifications and Registration	• Pharmacist registered and practicing with the GPhC • Pharmacy technician registered and practicing with the GPhC
Training and assessment	Pharmacists and Pharmacy Technicians must have completed training relevant to this condition to use this PGD. This training will be documented and overseen by the Get Real Health team. • All users must be familiar with the SPC for the products as well as BNF advice and any applicable national guidelines • All users must previously have used PGDs to supply medication • Must work in compliance with the SOPs of their own employer • All users must have appropriate indemnity in place to cover this service
Further training and responsibilities	• All users must be up to date with CPD requirements and appraisal • All users must be up to date with MHRA and safety alerts with regards to medical products • All users must understand the concepts of capacity and consent and be aware of the Mental Capacity Act 2005.

PGD Indication	Alternative treatment of uncomplicated recurrent lower urinary tract infection (cystitis) when nitrofurantoin is contraindicated or unsuitable in non-pregnant women aged 16-65 years
Inclusion criteria	Non-pregnant women aged 16-65 years Symptoms consistent with uncomplicated lower UTI (dysuria, urinary frequency, urinary urgency, suprapubic pain) Urine dipstick positive for nitrites and/or leucocytes History of recurrent UTI (≥ 2 in 6 months or ≥ 3 in 12 months) Nitrofurantoin contraindicated or unsuitable (e.g. allergy, renal impairment, pulmonary disease) Valid informed consent obtained

Exclusion criteria	Pregnancy - teratogenic in first trimester (folate antagonist); avoid throughout pregnancy Breastfeeding - relative contraindication; excreted in breast milk Blood dyscrasias or history of blood disorders Severe hepatic impairment Severe renal impairment (eGFR <15 ml/min) Known hypersensitivity to trimethoprim or excipients Concurrent methotrexate use (increased folate antagonism risk) Systemic symptoms or fever (signs of pyelonephritis - refer to GP) Haematuria Catheter-related UTI
Cautions	Folate deficiency - increased risk of blood dyscrasias; assess before treatment Elderly patients (≥65 years) - assess renal function and folate status Renal impairment: eGFR 15-30 ml/min - reduce dose or avoid Predisposition to folate deficiency (e.g. chronic alcoholism, malnutrition) Risk of hyperkalaemia, especially in renal impairment
Actions if patient is excluded or declines treatment	Advise on alternative treatment options and how these can be accessed. Document any advice given and the decision reached. Inform or refer to the GP as appropriate.

Details of the medicine

Name, form and strength of medicine	Trimethoprim 200mg tablets
Legal category	Prescription Only Medicine (POM)
Storage	Store below 25°C. Protect from light.
Route/method of administration	Oral - via mouth
Dose and frequency	200mg twice daily for 3 days
Quantity to be administered and/or supplied	6 tablets (3-day course)
Maximum or minimum treatment period	3 days maximum
Adverse effects	Common: nausea, vomiting, rash, pruritus Uncommon: blood dyscrasias (agranulocytosis, thrombocytopenia,

	megaloblastic anaemia), photosensitivity Rare: erythema multiforme, Stevens-Johnson Syndrome (SJS), toxic epidermal necrolysis (TEN)
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Yellow card reporting	Report suspected adverse effects via https://yellowcard.mhra.gov.uk and inform the GP as appropriate.
Records to be kept	Record the following information: • that valid informed consent was given • name of individual, address, date of birth and GP • name of healthcare practitioner • name and brand of medication • date of supply dose, form and route quantity supplied • advice given, including advice given if excluded or declines treatment • details of any adverse drug reactions and actions taken supplied via PGD Records should be signed and dated. All records should be clear, legible and contemporaneous. Electronic records can be made. A record of all individuals receiving treatment under this PGD should also be kept for audit purposes as follows: • Adults aged 18 years and over - keep records for 8 years • Children aged under 18 years - keep records until the 25th birthday (if the patient was 17 years when treatment was finished, keep records until the 26th birthday)

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This section must be signed by the superintendent or clinical lead responsible for this service.

Name of superintendent	Job title & name of	Signature	Date
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/ clinical lead	organisation		
	(GPhC: 2046322)		
Name of professional group signatory	Job title & name of organisation	Signature	Date
	(GPhC: 2046322)		

Signed on behalf of Get Real Health

Name	Qualification	Signature	Date
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